A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

22 April 2026

Dr Amila Fernando
Bairnsdale Family Practice

Dear Dr Fernando,

Re: Miss Rebecca Mauriks
DOB: 1977-08-13
Address: 40 Eighth Ave, RAYMOND ISLAND, VIC, 3880

Summary

- **Management of persistent morning faecal urgency and peripheral neuropathy.**
- **Escalation of amitriptyline dose for improved symptom control.**
- **Cessation of mesalazine and Metamucil with stable gastrointestinal status.**
- **Review scheduled in 12 months.**
- **No action required by you.**

I had the pleasure of conducting a review with Rebecca Mauriks (kellaway). Overall, her symptoms have improved, though she continues to experience morning faecal urgency. Reassuringly, she has not experienced any further episodes of faecal incontinence since our last consultation.

We reviewed her current medications and recent changes. Rebecca has ceased both mesalazine and Metamucil. As there has been no subsequent exacerbation of her bowel symptoms or change in stool consistency, I agree that these interventions were likely not contributing significantly to her clinical management, and she will remain off them.

Our discussion primarily focused on the management of her peripheral neuropathy, which continues to cause pain in her feet and ankles. She is currently taking 25 mg of amitriptyline and reports no adverse effects, such as xerostomia or somnolence. She believes the current dose provides some relief for her neurological symptoms, but there is scope for further improvement.

Impression and Plan

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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I have recommended a trial of dose escalation for her amitriptyline. This is intended to address the residual morning urgency while providing better analgesic coverage for her peripheral neuropathy. I have advised her to increase the dose to 37.5 mg (utilising one and a half 25 mg tablets) and, if tolerated, to proceed to 50 mg. I have provided prescriptions for both 25 mg and 50 mg tablets to facilitate this transition.

If she develops significant side effects at higher doses, I have instructed her to revert to the previously tolerated 25 mg dose. Given the chronic nature of her peripheral neuropathy, she will likely require long-term neuromodulatory therapy. I have scheduled a review in 12 months to monitor her progress, but I have encouraged her to maintain regular reviews with you for her ongoing scripts in the interim.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Prof Matthew Read

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